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Time taken	9 secs
Marks	0.00/80.00
Grade	0.00 out of 100.00

Question 1

Not answered

Marked out of 1.00

The current prevalence of heroin use in the UK is

Select one:

- ☐ 10%
- ☐ 5%
- ☐ 2-3%
- ☐ < 1%
- ☐ 20%

Your answer is incorrect.

Explanation:

The current prevalence of heroin use in the UK is <1%.

Ref: <https://www.ons.gov.uk/releases/drugmisuseinenglandandwalesyearendingmarch2020>

The correct answer is: < 1%

Question 2

Not answered

Marked out of 1.00

Approximately what percentage of patients taking benzodiazepines for more than a year develop dependence?

Select one:

- ☐ 80%
- ☐ 40%
- ☐ 15%
- ☐ 60%
- ☐ 10%

Your answer is incorrect.

Explanation:

Estimates suggest that few people become dependent on benzodiazepine with periods of use of less than 3 months. Between 3 and 12 months of use, 10-20% will become dependent, rising to 20-45% after periods longer than a year.

Ref: Adam R. Winstock, in Core Psychiatry (Third Edition), 2012. Pp. 439-462.

The correct answer is: 40%



Question 3

Not answered

Marked out of 1.00

How much of heroin does a typical dependent user take in a day?

Select one:

- ☐ 10-20 g per day
- ☐ 0.25-2.0 g per day
- ☐ 0.1-0.2 g per day
- ☐ 2.0-4.0 g per day
- ☐ 5.0-10.0 g per day

Your answer is incorrect.

Explanation:

Illicit heroin is sold as a brown or white powder in 'bags' or 'wraps', costing £50- £100 per gram, with a typical dependent user taking 0.25-2.0 g per day. It is most commonly consumed by smoking ('chasing') but is also taken orally, occasionally snorted, and parenterally by IV, IM, or subcutaneous routes. Street supplies are of variable purity (25-50% by volume). Occasionally, a particularly pure batch is associated with a series of deaths and overdoses from users used to a less concentrated form.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 618.

The correct answer is: 0.25-2.0 g per day

Question 4

Not answered

Marked out of 1.00

Ice is the street name for a particularly strong and pure form of

Select one:

- ☐ Amphetamine
- ☐ Cocaine
- ☐ Cannabis
- ☐ Phencyclidine
- ☐ Heroin

Your answer is incorrect.

Explanation:

Typically, amphetamines are used to **stimulate performance** and induce a **euphoric feeling** by students, truck drivers, athletes, soldiers, etc. **Oral, IV, and insufflation** (snorting) are possible routes of use. Many **synthetic forms** are now available. **Methamphetamine** ('crystal meth', 'whizz', 'ice') is a potent form that can be inhaled, smoked, or injected intravenously.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 617, 622-623.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 302.

The correct answer is: Amphetamine



Question 5

Not answered

Marked out of 1.00

Which of the following is an acute effect caused by LSD (Lysergic acid diethylamide)?

Select one:

- ☐ persistent psychosis
- ☐ post hallucinogenic perceptual disorder
- ☐ flashbacks
- ☐ persistent anxiety
- ☐ bad trips

Your answer is incorrect.

Explanation:

Acute harmful effects of LSD include behavioural toxicity (i.e. harm related to acting on beliefs such as having the ability to fly) and 'bad trips' (i.e. dissociation, fear of incipient madness, frightening perceptions). There is no risk of overdose, and physiological dependence and withdrawals do not occur. Chronic harmful effects include flashbacks even many years after consumption, post-hallucinogenic perceptual disorder, persistent psychosis, and persistent anxiety/depressive symptoms.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

The correct answer is: bad trips

Question 6

Not answered

Marked out of 1.00

Which of the following is a preferred maintenance treatment for opioid use disorder?

Select one:

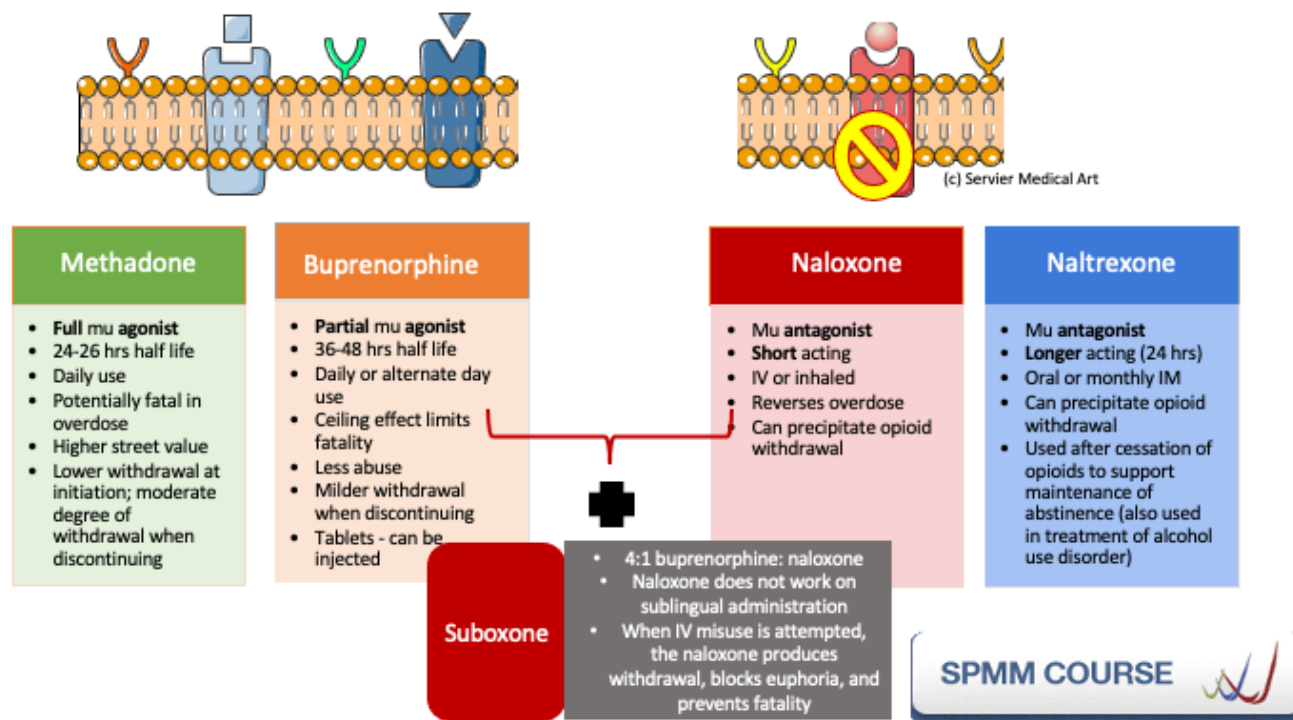
- ☐ Lofexidine
- ☐ Oral morphine
- ☐ Buprenorphine
- ☐ Naloxone
- ☐ LAAM

Your answer is incorrect.

Explanation:

Buprenorphine is considered to be a safe option to treat opioid use disorder with a very low potential for abuse, especially when used in combination with naloxone in a sublingual preparation (Suboxone).





Ref: Schuckit MA. Treatment of opioid-use disorders. New England Journal of Medicine. 2016 Jul 28;375(4):357-68.

Bell J, Strang J. Medication treatment of opioid use disorder. Biological psychiatry. 2020 Jan 1;87(1):82-8.

The correct answer is: Buprenorphine

Question 7

Not answered

Marked out of 1.00

Suboxone is a drug that contains

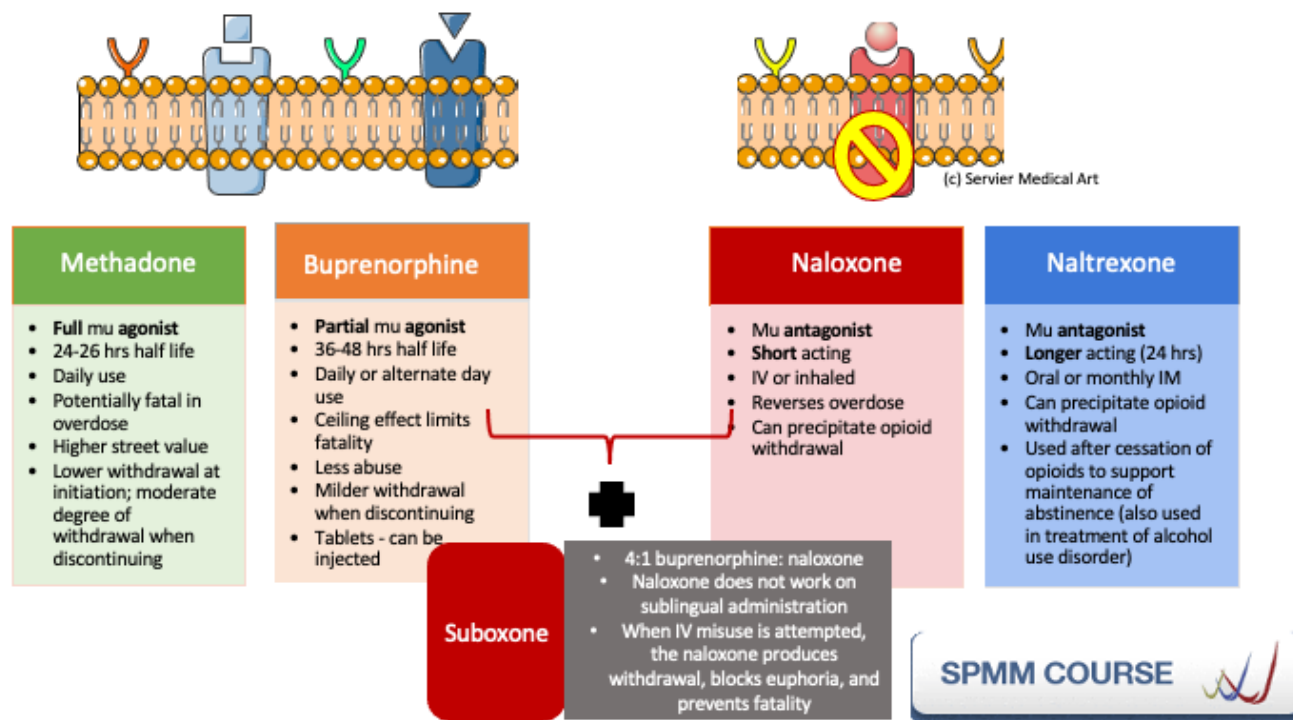
Select one:

- ☐ Buprenorphine and naloxone
- ☐ Buprenorphine and methadone
- ☐ Buprenorphine and naltrexone
- ☐ Methadone and naloxone
- ☐ Methadone and naltrexone

Your answer is incorrect.

Explanation:

Suboxone contains buprenorphine as well as the opioid antagonist naloxone to deter the abuse of prescribed buprenorphine via intravenous injection of the crushed tablets. Controlled trials suggest that buprenorphine and naloxone mixed at a 4:1 ratio in suboxone produces unpleasant withdrawal symptoms if taken intravenously by opioid users, but has no such effect when taken sublingually. However, the suboxone formulation still has potential to produce an opioid agonist "high" if injected by non-dependent persons. Suboxone has been approved by the European regulatory body for prescribed medications.



Ref: Schuckit MA. Treatment of opioid-use disorders. New England Journal of Medicine. 2016 Jul 28;375(4):357-68.

Bell J, Strang J. Medication treatment of opioid use disorder. Biological psychiatry. 2020 Jan 1;87(1):82-8.

The correct answer is: Buprenorphine and naloxone

Question 8

Not answered

Marked out of 1.00

Which drug lacks an evidence base for treating opioid withdrawal symptoms?

Select one:

- ☐ Buprenorphine
- ☐ Clonidine
- ☐ Methadone
- ☐ Chlordiazepoxide
- ☐ Lofexidine

Your answer is incorrect.

Explanation:

In the management of opioid withdrawal symptoms, there is a substantial evidence base for **three main types of pharmacotherapy** (all are effective in reducing withdrawal symptoms):

1. **Methadone** at tapered doses
2. **Buprenorphine**
3. **α_2 adrenergic agonists** (usually lofexidine; can consider clonidine)

The choice of agent may be guided by the patient's preference, desired duration of treatment, adverse effects and withdrawal severity.

Ref: Schuckit MA. Treatment of opioid-use disorders. New England Journal of Medicine. 2016 Jul 28;375(4):357-68.

Lingford-Hughes AR et al. BAP updated guidelines: evidence-based guidelines for the pharmacological management of substance abuse, harmful use, addiction and comorbidity: recommendations from BAP. Journal of Psychopharmacology. 2012 Jul;26(7):899-952.

The correct answer is: Chlordiazepoxide

Question 9

Not answered

Marked out of 1.00

Mr. Z has a history of using illicit drugs. He is observed to have a nasal septal perforation. Which of the following is the most likely offending drug?

Select one:

- ☐ Cannabis
- ☐ LSD
- ☐ Heroin
- ☐ Amphetamines
- ☐ Cocaine

Your answer is incorrect.

Explanation:

Chronic harmful effects of cocaine include necrosis of the nasal septum, fetal damage ('crack babies'), panic and anxiety disorders, persecutory delusions, and psychosis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 622.

The correct answer is: Cocaine



Question 10

Not answered

Marked out of 1.00

'Crack' is a pure form of which of the following street drugs?

Select one:

- ☐ Amphetamines
- ☐ LSD
- ☐ Heroin
- ☐ Cocaine
- ☐ Cannabis

Your answer is incorrect.

Explanation:

Street cocaine can be **freebased** (released free from the base e.g. sodium bicarbonate which is mixed with it) to produce a **purier** and **extremely potent form** called **crack** which can be smoked. In terms of rapidity of action and peak blood levels, it compares with IV use.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 622.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 303.

The correct answer is: Cocaine



Question 11

Not answered

Marked out of 1.00

Tolerance does not develop to which of the following features of opioid use?

Select one:

- ☐ Pupillary dilatation
- ☐ Sedation
- ☐ Constipation
- ☐ Euphoria
- ☐ Analgesic effect

Your answer is incorrect.

Explanation:

Tolerance to opioid-induced **constipation** does not usually develop with long-term use.

Ref: Nelson AD, Camilleri M. Chronic opioid induced constipation in patients with nonmalignant pain: challenges and opportunities. Therapeutic advances in gastroenterology. 2015 Jul;8(4):206-20.

The correct answer is: Constipation



Question 12

Not answered

Marked out of 1.00

The intervention with most consistent evidence base among all psychosocial interventions for cocaine users is

Select one:

- ☐ Vocational training
- ☐ Contingency management
- ☐ Problem solving therapy
- ☐ Motivational enhancement
- ☐ Residential rehab housing

Your answer is incorrect.

Explanation:

Psychosocial interventions are first-line treatment for cocaine use disorders. **Contingency management** (alone or in combination with community reinforcement or CBT) has been shown to have superior efficacy.

Ref: De Crescenzo F et al. Comparative efficacy and acceptability of psychosocial interventions for individuals with cocaine and amphetamine addiction: A systematic review and network meta-analysis. PLoS medicine. 2018 Dec 26;15(12):e1002715.

The correct answer is: Contingency management

Question 13

Not answered

Marked out of 1.00

The common side effects of benzodiazepines include which of the following?

Select one:

- ☐ Dizziness
- ☐ Gastrointestinal upset
- ☐ Disinhibition
- ☐ Headache
- ☐ Blurred vision

Your answer is incorrect.

Explanation:

The most common side effects of benzodiazepines are drowsiness, ataxia, and dizziness.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 671.

The correct answer is: Dizziness



Question 14

Not answered

Marked out of 1.00

Which one of the following is a specific benzodiazepine antagonist used in accident and emergency departments to reverse the effects of benzodiazepines?

Select one:

- ☐ Flumazenil
- ☐ Naloxone
- ☐ Naltrexone
- ☐ Saline infusion
- ☐ Apomorphine

Your answer is incorrect.

Explanation:

In contrast to barbiturates, benzodiazepines have a **larger margin of safety in overdoses**. The ratio of lethal dose to effective dose is about 200 to 1 or higher. When benzodiazepines are taken in **overdose with alcohol or other sedatives**, this can be **more serious; death** is usually due to **respiratory depression**. Flumazenil is a **specific benzodiazepine antagonist** used in emergency departments to reverse effects of benzodiazepines.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 299.

The correct answer is: Flumazenil

Question 15

Not answered

Marked out of 1.00

Which benzodiazepine has been particularly implicated in cases of 'date rape'?

Select one:

- ☐ Flunitrazepam
- ☐ Clonazepam
- ☐ Diazepam
- ☐ Lorazepam
- ☐ Temazepam

Your answer is incorrect.

Explanation:

Flunitrazepam is a short-acting, potent benzodiazepine seen particularly in dance settings with intoxicant and (probably apocryphal) aphrodisiac effects. It can produce impaired judgment and anterograde amnesia and is tasteless in solution. It has been implicated in cases of **date rape**.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 620.

The correct answer is: Flunitrazepam

Question **16**

Not answered

Marked out of 1.00

According to ICD-11, which term is used to denote a state in which alcohol has caused damage to a person's physical or mental health, without meeting criteria for dependence?

Select one:

- ☐ Normality
- ☐ Harmful pattern of use
- ☐ Compulsive use
- ☐ Abuse
- ☐ Hazardous use

Your answer is incorrect.

Explanation:

ICD-11 has a diagnostic code for 'harmful pattern of use' wherein the pattern of alcohol use has caused damage to a person's physical or mental health or has resulted in behaviour leading to harm to the health of others. In alcohol dependence, individuals use alcohol persistently despite harm and adverse consequences. Harm caused by such use may be similar, but alcohol dependence includes additional features of impaired ability to control use and increasing priority given to the substance use over other activities. Physiological features such as tolerance may also be present in dependence. If all diagnostic requirements for alcohol dependence are met, harmful pattern of use of alcohol should not be assigned.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Harmful pattern of use

Question 17

Not answered

Marked out of 1.00

The most commonly used illicit opioid in Europe (including the UK) is

Select one:

- ☐ Heroin
- ☐ Fentanyl
- ☐ Morphine
- ☐ Codeine
- ☐ Methadone

Your answer is incorrect.

Explanation:

In Europe, the most commonly used illicit opioid is heroin, which may be smoked, snorted, or injected. A range of synthetic opioids such as methadone, buprenorphine, and fentanyl are also misused. Opioids other than heroin reported by treatment entrants include misused methadone, buprenorphine, fentanyl, codeine, morphine, tramadol, and oxycodone. The five most populous countries in Europe, accounting for 62% of its population, contain three-quarters of its estimated high-risk opioid users (Germany, Spain, France, Italy, United Kingdom).

Ref: European Monitoring Centre for Drugs and Drug Addiction. European Drug Report 2019: Trends and Developments, Publications Office of the European Union, Luxembourg. 2019.

Alho H et al. Opioid-related deaths in Europe: strategies for a comprehensive approach to address a major public health concern. International Journal of Drug Policy. 2020 Feb 1;76:102616.

The correct answer is: Heroin

Question **18**

Not answered

Marked out of 1.00

Which of the following drugs is no longer used as patients developed prolonged QT intervals associated with fatal torsades de pointes?

Select one:

- ☐ Methadone
- ☐ Clonidine
- ☐ LAAM
- ☐ Buprenorphine
- ☐ Suboxone

Your answer is incorrect.

Explanation:

Levo-alpha-acetylmethadol (LAAM) is a synthetic mu receptor agonist that was used in opioid maintenance treatment. It was withdrawn from the market due to QT interval prolongation and life-threatening/fatal cardiac arrhythmias such as torsades de pointes.

Ref: Behzadi M et al. Opioids and cardiac arrhythmia: a literature review. Medical principles and practice. 2018;27(5):401-14.

The correct answer is: LAAM



Question 19

Not answered

Marked out of 1.00

Which of the following is an alpha-2 adrenergic agonist, licensed for the management of symptoms of opioid withdrawal?

Select one:

- ☐ Buprenorphine
- ☐ Naltrexone
- ☐ Naloxone
- ☐ Acamprosate
- ☐ Lofexidine

Your answer is incorrect.

Explanation:

Lofexidine is a centrally acting alpha 2 agonist. It is commonly prescribed for the mitigation of opioid withdrawal symptoms and to facilitate abrupt opioid discontinuation. Lofexidine is taken during the time of peak opioid withdrawal for up to 14 days.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Prescriber's Guide, 7th Edition. 2021. pp. 427.

The correct answer is: Lofexidine



Question **20**

Not answered

Marked out of 1.00

Mr.Z has presented to A and E with a history of ingesting an illicit drug. He believes that he can fly. Which of the following is the most likely offending drug?

Select one:

- ☐ Cannabis
- ☐ Heroin
- ☐ Ecstasy
- ☐ LSD
- ☐ Speed

Your answer is incorrect.

Explanation:

Acute harmful effects of LSD include behavioural toxicity (i.e. harm related to acting on beliefs such as having the ability to fly) and 'bad trips' (i.e. dissociation, fear of incipient madness, frightening perceptions). There is no risk of overdose, and physiological dependence and withdrawals do not occur. Chronic harmful effects include flashbacks even many years after consumption, post-hallucinogenic perceptual disorder, persistent psychosis, and persistent anxiety/depressive symptoms.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

The correct answer is: LSD

Question **21**

Not answered

Marked out of 1.00

Which one among the following drugs is recommended by NICE guidance as the first choice treatment for opioid dependence?

Select one:

- ☐ Methadone
- ☐ Buprenorphine
- ☐ Acamprosate
- ☐ Naloxone
- ☐ Naltrexone

Your answer is incorrect.

Explanation:

Most recent NICE guidelines for the management of opioid dependence (2007, update 2016) recommend **methadone** and **buprenorphine** as options for maintenance therapy. The decision about which drug to use should be made on a case-by-case basis. If both are equally suitable, **methadone** should be the first choice.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on methadone and buprenorphine for the management of opioid dependence. Updated edition 2016.

The correct answer is: Methadone

Question **22**

Not answered

Marked out of 1.00

Which of the following is a chronic harmful effect of cocaine?

Select one:

- ☐ Impaired judgement
- ☐ Arrhythmias
- ☐ Intense acute anxiety
- ☐ Hypertension
- ☐ Necrosis of nasal septum

Your answer is incorrect.

Explanation:

Chronic harmful effects of cocaine include necrosis of the nasal septum, fetal damage ('crack babies'), panic and anxiety disorders, persecutory delusions, and psychosis. Cocaine is not associated with classical dependence, but a minority of users will consume in a regular 'compulsive' pattern.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 622.

The correct answer is: Necrosis of nasal septum

Question **23**

Not answered

Marked out of 1.00

The term cold turkey is commonly used to refer to

Select one:

- ☐ Cannabis withdrawal
- ☐ Amphetamine withdrawal
- ☐ Alcohol withdrawal
- ☐ Cocaine withdrawal
- ☐ Opioid withdrawal

Your answer is incorrect.

Explanation:

The term 'cold turkey' is often used for the opioid withdrawal syndrome and refers particularly to the piloerection or gooseflesh experienced during withdrawal. Other opioid withdrawal symptoms include dysphoric mood, nausea or vomiting, muscle aches, lacrimation or rhinorrhea, pupillary dilation, diarrhoea, yawning, fever, insomnia, hypertension, and tachycardia. Persons with opioid dependence seldom die from opioid withdrawal unless they have a severe pre-existing physical illness such as cardiac disease. Residual symptoms – such as insomnia, bradycardia, temperature dysregulation, and a craving for opioids – can persist for months after withdrawal.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 292.

The correct answer is: Opioid withdrawal

Question **24**

Not answered

Marked out of 1.00

Which of the following drugs and its street name is incorrectly matched?

Select one:

- ☐ Heroin - brown sugar
- ☐ LSD - acid
- ☐ Amphetamine - speed
- ☐ Phencyclidine - dope
- ☐ Freebase cocaine - crack

Your answer is incorrect.

Explanation:



Conventional name	Street names
Amphetamine	Billy/Whizz, Speed, Sulph
Cannabis	Bud, Chronic, Dope, Ganja, Grass, Green, Hash, Hashish, Hemp, Herb, Kush, Marijuana, Mary Jane, Pot, Purple Haze, Reefer, Sinsemilla, Skunk (potent), Trees, Weed
Cocaine	Bernice, Blow, C, Charlie, Coke, Crack (freebase), Dust, Flake, Line, Nose Candy, Rock, Sneeze, Sniff, Snow, Toot, White, Yayo
Diazepam	Vallies (from Valium)
Heroin	Big H, Black Tar, Boy, Brown Sugar, China White, Dope, Dragon, Gear, H, Horse, Junk, Mexican Brown, Mug, Scag, Skag, Skunk, Smack, Thunder
Ketamine	Cat Valium, Green K, Honey Oil, Jet, Ket, Kit Kat, Purple, Special K, Special LA Coke, Super Acid, Super C, Vitamin K
LSD	Acid, Battery Acid, Blotter, California Sunshine, Cid, Doses, Dots, L, Looney Toons, Lucy/in the Sky with Diamonds, Superman, Tabs, Window Pane, Yellow Sunshine
Methamphetamine	Chalk, Crank, Crissy, Cristy, Crystal, Crystal Meth, Glass, Go, Ice, Meth, Shards, Tina, Tweak, Whizz
MDMA	Adam, Beans, Candy, Clarity, Dancing Shoes, E, Ecstasy, Eccies, Happy Pill, Hug, Hug Drug, Love Drug, Lover's Speed, Molly, Moon Rocks, Rolls, Scooby Snacks, X, XTC
PCP	Angel Dust, Embalming fluid, Hog, Love boat, Ozone, Rocket fuel, Superweed, Wack, Wet (a marijuana joint dipped in PCP)
Psilocybin mushrooms	Blue Meanies, Boomers, Buttons, Caps, Cubes, Liberties, Liberty Caps, Magic Mushrooms, Magics, Mushies, Shrooms
Temazepam	Jellies

Adapted from Oxford Handbook of Psychiatry, 2019. p. 617.

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Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 617.

The correct answer is: Phencyclidine - dope

Question 25

Not answered

Marked out of 1.00

The most common cause of death in benzodiazepine overdose is

Select one:

- ☐ Liver failure
- ☐ Respiratory depression
- ☐ Myoglobinuria
- ☐ Renal failure
- ☐ Myocardial infarction

Your answer is incorrect.

Explanation:

In contrast to barbiturates, benzodiazepines have a **larger margin of safety in overdoses**. The ratio of lethal dose to effective dose is about 200 to 1 or higher. When benzodiazepines are taken in **overdose with alcohol or other sedatives**, this can be **more serious; death** is usually due to **respiratory depression**.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 299.

The correct answer is: Respiratory depression

Question 26

Not answered

Marked out of 1.00

A pregnant woman who is currently using heroin requests to undergo opioid detoxification. Which of the following is the safest period to undertake opioid detoxification in a pregnant woman?

Select one:

- ☐ Second trimester
- ☐ Third trimester
- ☐ As soon as possible in any trimester
- ☐ First trimester
- ☐ Should never be attempted when pregnant

Your answer is incorrect.

Explanation:

Women can present with opioid dependency at any stage in pregnancy, and stabilisation on substitute methadone is the treatment of choice. Detoxification in the first trimester is contraindicated due to the risk of spontaneous abortion and in the third trimester it is associated with preterm delivery, foetal distress, and stillbirth. If detoxification is requested, this is most safely achieved in the second trimester but should only be supervised by specialists with the appropriate competencies and with careful monitoring for any evidence of instability. Detoxification should be prescribed in small frequent decrements (e.g. 2-3mg of methadone every 3-5 days). Enforcing detoxification is contraindicated.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 498.

The correct answer is: Second trimester

Question **27**

Not answered

Marked out of 1.00

Which of the following is a common side effect of clonidine?

Select one:

- ☐ Dysphoria
- ☐ Sedation
- ☐ Diarrhoea
- ☐ Hypersalivation
- ☐ Hypertension

Your answer is incorrect.

Explanation:

The most common adverse effects associated with clonidine are dry mouth and eyes, fatigue, sedation, dizziness, nausea, hypotension, and constipation.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 695-696.

The correct answer is: Sedation

Question 28

Not answered

Marked out of 1.00

With respect to naltrexone use in opioid dependence, which of the following is true?

Select one:

- ☐ Naltrexone implants are licensed in the UK for reducing opioid use
- ☐ Dysphoria, depression and insomnia are seldom experienced with naltrexone
- ☐ Naltrexone can be initiated irrespective of the amount of current opioid use
- ☐ Naltrexone is superior to methadone in relapse prevention among opioid users
- ☐ Naltrexone's effectiveness as a treatment for relapse prevention in opioid misusers is inconclusive

Your answer is incorrect.

Explanation:

Evidence for the effectiveness of naltrexone as a treatment for relapse prevention in opioid misusers has been inclusive. However, naltrexone has been found by NICE to be a cost-effective treatment strategy in aiding abstinence from opioid misuse for those who prefer an abstinence programme, are fully informed of the potential adverse effects and benefits of treatment, are highly motivated to remain on treatment, and have a partner supporting concordance. A naltrexone implant, not currently licensed in the UK, may also have a role to play in reducing opioid use in a motivated population of patients. People taking naltrexone often experience adverse effects of unease (dysphoria), depression, and insomnia. Naltrexone has the propensity to cause a severe withdrawal reaction in patients who are either currently taking opioid drugs or who were previously taking opioid drugs and there has not been a sufficient 'wash-out' period.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 496.

The correct answer is: Naltrexone's effectiveness as a treatment for relapse prevention in opioid misusers is inconclusive

Question **29**

Not answered

Marked out of 1.00

In an average user, LSD can be detected in the urine for up to

Select one:

- ☐ 4 days
- ☐ 7 days
- ☐ 1 day
- ☐ 10 days
- ☐ LSD cannot be detected in urine

Your answer is incorrect.

Explanation:

LSD can be detected in the urine for up to 4 days.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 533.

The correct answer is: 4 days



Question 30

Not answered

Marked out of 1.00

The risk of seizures associated with bupropion extended release use is estimated as

Select one:

- ☐ 1 in 100
- ☐ 1 in 5000
- ☐ 1 in 10
- ☐ 1 in 1000
- ☐ 1 in 500

Your answer is incorrect.

Explanation:

The risk of seizures associated with the anti-smoking and antidepressant agent bupropion is said to be high, especially with the instant release version as the risk is dose-dependent, with three times a day dosing in patients leads to accidental overdose when patients take their tablets too close together, leading to higher plasma levels and higher seizure risk. With sustained release and extended release versions, this has come down to an extent that some recent studies put clomipramine, escitalopram etc. rather than bupropion as the highest offender (Steiner et al., 2018; . Finkelstein et al., 2018).

At present, the seizure inducing potential is said to be dose dependent. Following changes to dosing recommendations, seizure risk is reduced from 0.6% to 0.4% for regular 3-times a day dosing and to 0.1% for extended-release preparations (Craig et al., 2020).

Ref:

1. Steinert T, FrÄ¶scher W. Epileptic Seizures Under Antidepressive Drug Treatment: Systematic Review. *Pharmacopsychiatry*, 2018;51(4):121-135.
2. Finkelstein Y, Macdonald EM, Li P, et al. Second-generation anti-depressants and risk of new-onset seizures in the elderly. *Clin Toxicol (Phila)*, 2018;56(12):1179-1184.

3. Craig DP, Osborne C. Risk of seizures with antidepressants: what is the evidence?. Drug and Therapeutics Bulletin. 2020 Sep 1;58(9):137-40.

The correct answer is: 1 in 1000



Question **31**

Not answered

Marked out of 1.00

Untreated heroin withdrawal symptoms typically reach their peak within how many hours after the last dose?

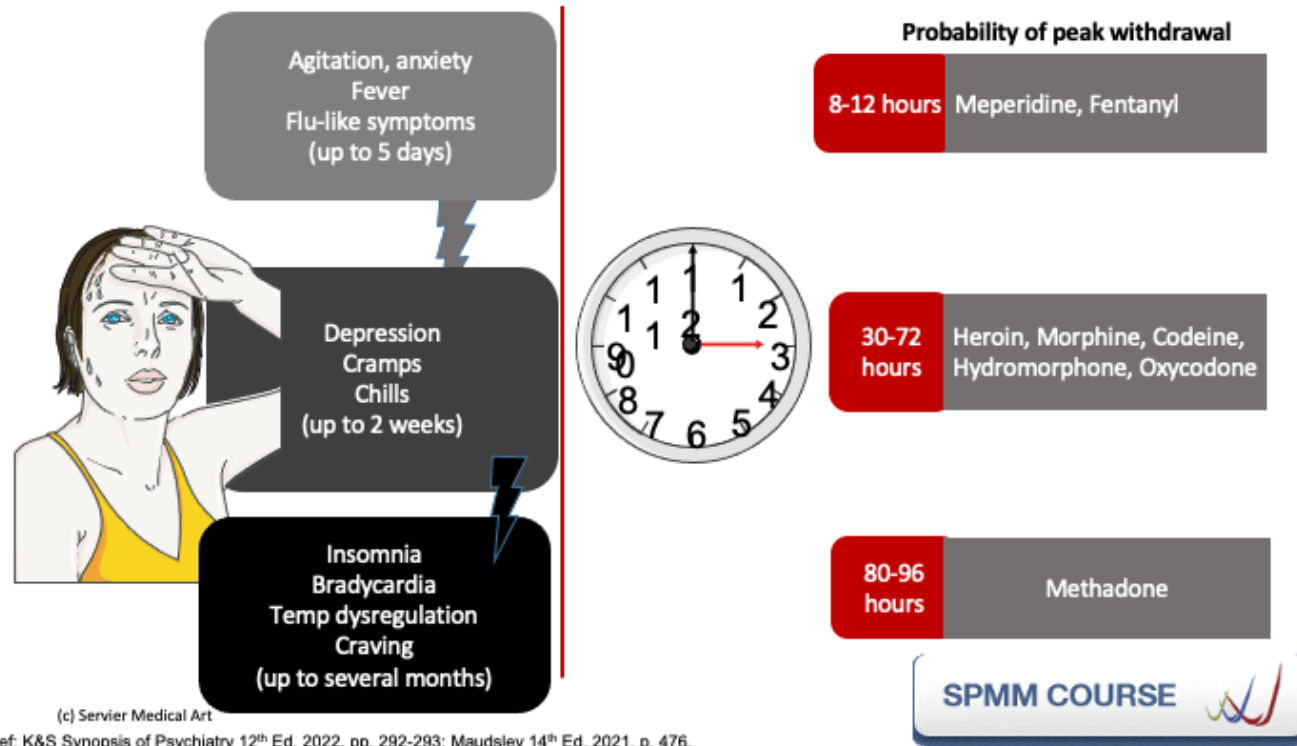
Select one:

- ☐ 12-24 hours
- ☐ 24-36 hours
- ☐ 32-72 hours
- ☐ 6-12 hours
- ☐ 72-120 hours

Your answer is incorrect.

Explanation:

Opioid withdrawal is rarely fatal in a healthy adult. Morphine and heroin withdrawal syndromes usually begin approximately 6 hours after the last dose, reach their peak at 32-72 hours, and subside during the next 7 to 10 days. Untreated methadone withdrawal typically reaches its peak between 4 and 6 days and symptoms do not subside for 10-12 days. However, insomnia, bradycardia, temperature dysregulation, and craving for opioids can persist for months after withdrawal.



Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 292-293.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 476.

The correct answer is: 32-72 hours

Question **32**

Not answered

Marked out of 1.00

Mr. X has been consuming a high dose of cannabis for a long time. How long will it be present in his urine?

Select one:

- ☐ 3 days
- ☐ 10 days
- ☐ 24 hours
- ☐ 7 days
- ☐ 4 weeks

Your answer is incorrect.

Explanation:

In individuals with chronic, heavy use, cannabis will be detectable in the urine for up to 4 weeks after use has ceased.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 631.

The correct answer is: 4 weeks



Question **33**

Not answered

Marked out of 1.00

For cessation of smoking, nicotine replacement therapy should be used for an initial period of

Select one:

- ☐ 18-22 weeks
- ☐ 4-8 weeks
- ☐ 2-4 weeks
- ☐ 8-12 weeks
- ☐ 1-2 weeks

Your answer is incorrect.

Explanation:

The initial treatment period for nicotine replacement therapy (NRT) is 8-12 weeks. NRT may be continued for up to 9 months if cravings persist. The most effective method of managing tobacco withdrawal symptoms is with combination NRT (i.e. a patch and oral product).

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. pp. 388-390.

The correct answer is: 8-12 weeks



Question **34**

Not answered

Marked out of 1.00

Which class of drugs is most likely to be abused by athletes and bodybuilders seeking a competitive advantage?

Select one:

- ☐ hallucinogens
- ☐ volatile solvents
- ☐ opioids
- ☐ anabolic steroids
- ☐ benzodiazepines

Your answer is incorrect.

Explanation:

Some **bodybuilders** and **athletes** abuse anabolic steroids (prescription-only medicines such as nandrolone and stanozolol) as an aid to **seeking a competitive advantage** or, more rarely, for their **euphoriant** effects alone. They produce increased muscle mass and strength, with increased training time and reduced recovery time, as well as euphoriant effects and a sense of increased energy levels.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 628.

The correct answer is: anabolic steroids

Question 35

Not answered

Marked out of 1.00

'Roid rage' is an acute emotional instability associated with the use of

Select one:

- ☐ benzodiazepines
- ☐ hallucinogens
- ☐ corticotrophins
- ☐ stimulants
- ☐ anabolic steroids

Your answer is incorrect.

Explanation:

Use of anabolic steroids is associated with **physical health problems** including hypertension, hypogonadism, gynaecomastia, amenorrhea, liver damage, impotence, male pattern baldness, and infection risks (if injected). **Associated mental health problems** include acute emotional instability (sometimes known as 'roid rage'), increased aggressiveness, persecutory/grandiose delusions, depressive illness, and chronic fatigue.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 628.

The correct answer is: anabolic steroids

Question **36**

Not answered

Marked out of 1.00

Kinaesthetic hallucinations are reported in cases with

Select one:

- ☐ Nicotine withdrawal
- ☐ Benzodiazepine withdrawal
- ☐ Cannabis withdrawal
- ☐ Amphetamine withdrawal
- ☐ Cocaine withdrawal

Your answer is incorrect.

Explanation:

Kinaesthetic hallucinations affect the muscles and joints, and the patient feels that their limbs are being twisted, pulled, or moved. Vestibular sensations such as sinking in the bed or flying through the air can also be hallucinated and are best regarded as a variant of kinaesthetic hallucinations, occurring in organic states (most commonly delirium tremens). Kinaesthetic or vestibular hallucinations can occur in organic states such as alcohol intoxication and during benzodiazepine withdrawal and may also occur in the absence of any abnormality (for example, after a week of sailing an undulating feeling may persist for a few days).

Ref: Casey P, Kelly B. Fish's clinical pathology: Signs and symptoms in psychiatry, 4th Edition. 2019. p. 34.

The correct answer is: Benzodiazepine withdrawal



Question **37**

Not answered

Marked out of 1.00

Which atypical antidepressant is recommended by NICE for smoking cessation?

Select one:

- ☐ Maprotiline
- ☐ Nefazadone
- ☐ Trazadone
- ☐ Bupropion
- ☐ Nomifensine

Your answer is incorrect.

Explanation:

First-line stop-smoking medications recommended by NICE and licensed in the UK are nicotine replacement therapy ('NRT', including gum, transdermal patches, inhalator, nasal spray, sublingual tablets, mouth spray, lozenges, and oral strips), varenicline, and bupropion. Bupropion is an antidepressant with dopaminergic and adrenergic actions and is additionally an antagonist at the nicotinic acetylcholine receptor.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 503-506.

The correct answer is: Bupropion

Question **38**

Not answered

Marked out of 1.00

Which of the following statements about caffeine is incorrect?

Select one:

- ☐ Caffeine acts as an antagonist of the adenosine receptors
- ☐ Caffeine is a methylxanthine
- ☐ Caffeine does not readily cross the blood-brain barrier
- ☐ The half-life of caffeine in the human body is 3 - 10 hours
- ☐ Caffeine at high doses can affect dopamine and noradrenergic neurons

Your answer is incorrect.

Explanation:

Caffeine is a methylxanthine more potent than theophylline. Caffeine's half-life in the human body is 3 - 10 hours. Caffeine readily crosses the blood-brain barrier and acts primarily as an antagonist of the adenosine receptors. At high doses, caffeine can affect dopamine and noradrenergic neurons. Single low to moderate doses of caffeine (i.e. 20 to 200 mg) increase alertness, concentration, energy, and motivation. 300 to 800 mg produces unpleasant anxiety and nervousness. Caffeine causes global cerebral vasoconstriction and a reduction in cerebral blood flow. Tolerance does not develop to these vasoconstrictive effects. Blood flow shows a rebound increase after withdrawal from caffeine.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 317.

The correct answer is: Caffeine does not readily cross the blood-brain barrier

Question 39

Not answered

Marked out of 1.00

Which of the following drugs is associated with an amotivational syndrome?

Select one:

- ☐ Opioids
- ☐ Mushrooms
- ☐ Cannabis
- ☐ Ketamine
- ☐ MDMA

Your answer is incorrect.

Explanation:

Chronic harmful effects of cannabis include dysthymia, anxiety/depressive illnesses, and the disputed 'amotivational syndrome' (possibly representing a combination of chronic intoxication in a heavy user and a long half-life).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 627.

The correct answer is: Cannabis



Question 40

Not answered

Marked out of 1.00

A 53-year-old man has a long-standing history of illicit drug use and has been recently diagnosed with lung cancer. Which of the following agents is potentially associated with carcinoma of lung?

Select one:

- ☐ LSD
- ☐ Heroin
- ☐ Cocaine
- ☐ Cannabis
- ☐ Speed

Your answer is incorrect.

Explanation:

Cannabis is often smoked with **tobacco**, so all the **health risks** associated with tobacco apply in those cases. The tendency of cannabis smokers to inhale deeply and to retain smoke in the lungs for as long as possible will **exacerbate** these risks. There are no reports of fatal OD. Acute harmful effects include mild paranoia, panic attacks, and accidents associated with delayed reaction time. Chronic harmful effects of cannabis include dysthymia, anxiety/depressive illnesses, and the disputed 'amotivational syndrome' (possibly representing a combination of chronic intoxication in a heavy user and a long half-life).

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 626-627.

The correct answer is: Cannabis

Question **41**

Not answered

Marked out of 1.00

Flashback phenomenon is commonly reported long after ingesting which of the following substances?

Select one:

- ☐ Psychotropic medication
- ☐ Nicotine
- ☐ Hallucinogens
- ☐ Psychostimulants
- ☐ Cocaine

Your answer is incorrect.

Explanation:

Long after ingesting a hallucinogen, a person can experience flashbacks of hallucinogenic symptoms. Hallucinogen persisting perception disorder features as a diagnostic category in the DSM-5 and ICD-11. This condition is characterised by perceptual distortions, hallucinations, and other experiential phenomena reminiscent of prior substance use. Flashbacks can be triggered by emotional stress, sensory deprivation (e.g., monotonous driving), or use of a psychoactive substance, such as alcohol or marijuana.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 322.

Vis PJ et al. On Perception and Consciousness in HPPD: A Systematic Review. Frontiers in neuroscience. 2021:966.

The correct answer is: Hallucinogens

Question **42**

Not answered

Marked out of 1.00

Mr. Z presents to the A and E with rashes around his nose and mouth and an unusual breath odour. On examination, he shows signs of ocular and oropharyngeal irritation. What class of substances is most likely to have produced this presentation?

Select one:

- ☐ Hallucinogens
- ☐ Alcohol
- ☐ Opioids
- ☐ Stimulants
- ☐ Inhalants

Your answer is incorrect.

Explanation:

Recent inhalant users may have rashes around the nose and mouth; unusual breath odours; residue of the inhalant substances on the face, hands, or clothes; and irritation of the eyes, throat, lungs, and nose. Adolescents (ages 12 to 17) are more likely to use inhalants than individuals in other age groups. Inhalant effects appear within 5 minutes and can last for 30 minutes to several hours. Inhalants generally act as CNS depressants. Signs of intoxication include nausea, anorexia, nystagmus, depressed reflexes, diplopia, stupor, and unconsciousness. The concentrations of many inhalant substances in the blood increase when combined with alcohol. Most persons probably use inhalants for a short time without developing dependence. Nonetheless, dependence on inhalants does occur. Tolerance can develop and a withdrawal syndrome can accompany the cessation of inhalant use.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. pp. 325-327.

The correct answer is: Inhalants

Question 43

Not answered

Marked out of 1.00

Smoking does not induce the metabolism of

Select one:

- ☐ Fluvoxamine
- ☐ Haloperidol
- ☐ Lithium
- ☐ Olanzapine
- ☐ Clozapine

Your answer is incorrect.

Explanation:

Smoking **lowers the plasma levels** of some drugs(e.g. agomelatine, some benzodiazepines, chlorpromazine, clozapine, duloxetine, fluphenazine, fluvoxamine, haloperidol, olanzapine, trazodone, tricyclic antidepressants). This can affect both **efficacy** and **side effects**. Lithium is renally cleared without undergoing any hepatic metabolism.

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. pp. 385-386.

The correct answer is: Lithium



Question **44**

Not answered

Marked out of 1.00

A 'bad trip' is an acute adverse effect of

Select one:

- ☐ Cocaine
- ☐ LSD
- ☐ Cannabis
- ☐ Amphetamines
- ☐ Heroin

Your answer is incorrect.

Explanation:

Acute harmful effects of LSD include behavioural toxicity (i.e. harm related to acting on beliefs such as having the ability to fly) and 'bad trips' (i.e. dissociation, fear of incipient madness, frightening perceptions). For all hallucinogens, acute psychotic features should be managed by maintenance of a safe environment with expectation of resolution.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

The correct answer is: LSD

Question **45**

Not answered

Marked out of 1.00

Which of the following drugs has shared amphetamine-like stimulant effects and LSD-like hallucinogen effects simultaneously?

Select one:

- ☐ Cocaine
- ☐ MDMA
- ☐ Methylphenidate
- ☐ Psilocybin
- ☐ Angel dust

Your answer is incorrect.

Explanation:

MDMA (ecstasy) is a substituted amphetamine that produces subjective effects resembling those of amphetamine and LSD (lysergic acid diethylamide). Therefore, it has both stimulant and hallucinogenic properties.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 303.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 623.

The correct answer is: MDMA



Question 46

Not answered

Marked out of 1.00

Of the following, the drug that is not licensed in UK for smoking cessation is

Select one:

- ☐ Methamphetamine
- ☐ Nicotine patches
- ☐ Nicotine inhalators
- ☐ Varenicline
- ☐ Bupropion

Your answer is incorrect.

Explanation:

First-line stop-smoking medications recommended by NICE and licensed in the UK are **nicotine replacement therapy** (NRT; including gum, transdermal patches, inhalator, nasal spray, sublingual tablets, mouth spray, lozenges, and oral strips), **varenicline**, and **bupropion**.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 503-506.

The correct answer is: Methamphetamine



Question 47

Not answered

Marked out of 1.00

Which of the following practices of injecting opioids is associated with Candida endophthalmitis?

Select one:

- ☐ Rotating injection sites
- ☐ Using sterile water to inject
- ☐ Sharing needles
- ☐ Using lemon juice to reconstitute
- ☐ Injecting in the direction of blood flow

Your answer is incorrect.

Explanation:

In tertiary eye care settings, retrospective studies indicate that between 70% and 78% of endogenous fungal endophthalmitis cases were caused by intravenous drug use. It has been found that dilution of heroin or cocaine with lemon juice can cause endophthalmitis caused by Candida (Proulx and Tousignant, 2021).

Harm reduction/safer injecting advice:

- Do **not** use opioids/opiates while **alone**
- Do **not** use in **combination** with other drugs
- **Avoid IV** route
- If injecting, always **inject with**, not against, **blood flow**
- **Rotate** injection sites
- **Avoid** injecting into the **neck, groin, or breast**
- **Avoid** injection into **infected areas**

- Ensure **complete dissolution** before injection – or else **emboli** can occur
- Use **new sterile needles** and **syringes** on each occasion
- Use **sterile water** (water from running cold kitchen tap is the closest); **avoid lemon juice**
- **Never share needles** or **equipment** with others

Ref: Proulx V, Tousignant B. Drugs of abuse and ocular effects. Clinical and experimental optometry. 2021 Jul 4;104(5):567-78.

Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 618-619.

The correct answer is: Using lemon juice to reconstitute



Question **48**

Not answered

Marked out of 1.00

Which of the following is a main side effect of varenicline?

Select one:

- ☐ Nausea
- ☐ Headache
- ☐ Agitation
- ☐ Dizziness
- ☐ Confusion

Your answer is incorrect.

Explanation:

Varenicline is a nicotinic receptor partial agonist. It is given orally and is the most effective cessation pharmacotherapy in the general population. In a head-to-head trial, varenicline was found to be more effective than bupropion, the nicotine patch, or placebo for helping people with or without a mental health diagnosis to stop smoking. Common side effects of varenicline include nausea, sleep disturbance, and vivid dreams.

Ref: Taylor DM, Gaughran F, Pillinger T. The Maudsley Practice Guidelines for Physical Health Conditions in Psychiatry. 2021. p. 390.

The correct answer is: Nausea



Question 49

Not answered

Marked out of 1.00

A 45-year-old man with bipolar disorder has acutely increased his caffeine consumption. According to current evidence, it is clear that he is at high risk of developing a

Select one:

- ☐ Depressive episode
- ☐ Manic episode
- ☐ Hypomanic episode
- ☐ Mixed episode
- ☐ None of the listed options

Your answer is incorrect.

Explanation:

The literature examining clinical effects of caffeine in patients with bipolar disorder is not conclusive. Acute increases in caffeine consumption may precede the occurrence of manic episodes in patients with bipolar disorder, potentially through a direct stimulant effect, affecting sleep patterns and/or the metabolism of lithium or other medications, although increases in caffeine intake could also be a consequence of an ongoing manic relapse or a prodromal sign. Further research is needed to determine whether caffeine use impacts the long-term prognosis of bipolar disorder.

Ref: Frigerio S et al. The impact of caffeine consumption on clinical symptoms in patients with bipolar disorder: A systematic review. *Bipolar Disorders*. 2021 May;23(3):241-51.

The correct answer is: None of the listed options

Question 50

Not answered

Marked out of 1.00

Mrs. Z has a history of abusing illicit drugs. She became pregnant and has now given birth to a boy. He was fine for first 24 hours after birth. However, he is now irritable with frequent yawning, sneezing, loose stools and fever. Tremors and jittery movements, along with high-pitched cries have been observed. Which of the following drugs when used by mothers during pregnancy can present with these withdrawal symptoms in the neonates?

Select one:

- ☐ Alcohol
- ☐ Nicotine
- ☐ Cocaine
- ☐ Cannabis
- ☐ Opioids

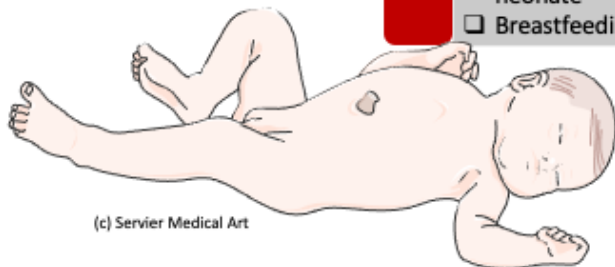
Your answer is incorrect.

Explanation:

Features and Management

Neonatal Opioid Withdrawal/Neonatal Abstinence Syndrome (NAS)

- ☐ Usually begins **24-48 hours** after birth, depending on the time of last dose; signs may not become apparent in some until later
- ☐ **Hyperirritability**, gastrointestinal dysfunction, respiratory distress, and vague autonomic symptoms (e.g., yawning, sneezing, mottling, fever)
- ☐ Tremors and jittery movements, **high-pitched cries** (characteristic), increased muscle tone, and irritability are common
- ☐ Normal reflexes may be exaggerated
- ☐ **Loose stools** are common, leading to possible electrolyte imbalances and diaper dermatitis
- ☐ Methadone withdrawal symptoms typically appear after 48 hours but can be delayed for 7-10 days
- ☐ Symptoms are milder with buprenorphine withdrawal
- ☐ Ensure access to skilled neonatal paediatric care to monitor and treat the neonate
- ☐ Breastfeeding may reduce the severity



Ref: Maudsley Prescribing Guidelines, 14th Ed. 2021. p. 499

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Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 499.

The correct answer is: Opioids



Question **51**

Not answered

Marked out of 1.00

You are called to A and E to see a young American tourist who has taken phencyclidine (PCP). Which of the following is least likely to be present?

Select one:

- ☐ Vertical or horizontal nystagmus
- ☐ Seizures
- ☐ Numbness or diminished responsiveness to pain
- ☐ Dysarthria with muscle rigidity
- ☐ Gait disturbances

Your answer is incorrect.

Explanation:

PCP is a hallucinogen rarely seen in the UK, except as a contaminant of other drugs. It may be smoked, snorted, taken orally, or, more rarely, parenterally. People who have just taken PCP are frequently uncommunicative, appear to be oblivious, and report active fantasy production. PCP intoxication can cause vertical or horizontal nystagmus, numbness or diminished responsiveness to pain, ataxia, dysarthria, muscle rigidity, and repeated episodes of vomiting. Occasionally, users become belligerent, irrationally assaultive, suicidal, or homicidal. Seizures are very rare.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 624.

Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 320.

The correct answer is: Seizures

Question 52

Not answered

Marked out of 1.00

Which of the following can reduce the harm associated with injecting opioids?

Select one:

- ☐ Injecting in the direction of blood flow
- ☐ Injecting into lymph nodes
- ☐ Injecting in the peritoneum to avoid clotting
- ☐ Using vinegar to reconstitute
- ☐ Seeking out isolated and quiet places to inject

Your answer is incorrect.

Explanation:

Harm reduction/safer injecting advice:

- Do **not** use opioids/opiates while **alone**
- Do **not** use in **combination** with other drugs
- **Avoid IV** route
- If injecting, always **inject with**, not against, **blood flow**
- **Rotate** injection sites
- **Avoid** injecting into the **neck, groin, or breast**
- **Avoid** injection into **infected areas**
- Ensure **complete dissolution** before injection – or else **emboli** can occur
- Use **new sterile needles** and **syringes** on each occasion

- Use **sterile water** (water from running cold kitchen tap is the closest)
- **Never share needles or equipment** with others

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 619.

The correct answer is: Injecting in the direction of blood flow



Question 53

Not answered

Marked out of 1.00

Following smoking cessation, women and heavy smokers are at greatest risk for which of the following?

Select one:

- ☐ Hypercholesterolaemia
- ☐ Hypertension
- ☐ Weight gain
- ☐ Diabetes
- ☐ Renal failure

Your answer is incorrect.

Explanation:

The reported **average increased weight gain** 10 years after quitting smoking is 5kg (compared with continuing smoking). Greater weight gain is seen among **heavier smokers** and those with **higher BMI before quitting**. Those especially **prone** to post-cessation weight gain include smokers of **low socioeconomic status, women**, and smokers who **eat when under emotional stress** or who **eat for comfort**.

Ref: Kos K. Smoking cessation, weight gain, and cardiovascular risk. The Lancet Diabetes & Endocrinology. 2020 Feb 1;8(2):93-5.

The correct answer is: Weight gain

Question **54**

Not answered

Marked out of 1.00

Delirium tremens occurs in what percentage of episodes of alcohol withdrawal?

Select one:

- ☐ 1-2%
- ☐ 20%
- ☐ 5%
- ☐ 10%
- ☐ 25%

Your answer is incorrect.

Explanation:

Delirium tremens (DTs) is an acute confusional state secondary to alcohol withdrawal. It is often preceded by withdrawal symptoms; seizure may herald its onset. It occurs in approximately 5% of episodes of alcohol withdrawal. Risk factors for DTs include a long history of alcohol dependence, past history of DTs, being older, medically compromised, and poor nutritional state. DTs are characterised by a range of clinical symptoms including autonomic hyperactivity, marked agitation, clouding of consciousness, perceptual disturbances, and paranoid delusions. Mortality is 1-15%.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 590-591.

Sinclair JM, O'Neill A. Update on the management of alcohol use disorders. BJPsych Advances. 2020 Mar;26(2):82-91.

The correct answer is: 5%

Question 55

Not answered

Marked out of 1.00

In Wernicke encephalopathy the classic triad of ataxia, confusion and ophthalmoplegia is seen in

Select one:

- ☐ 70%
- ☐ 1%
- ☐ 10%
- ☐ 80%
- ☐ 50%

Your answer is incorrect.

Explanation:

Wernicke encephalopathy is an acute condition characterized by the classic triad of confusion, ataxia, and ophthalmoplegia. However, this triad is only seen in 10% of cases; confusion is present in approximately 80% of cases.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 606.

The correct answer is: 10%



Question 56

Not answered

Marked out of 1.00

In the UK, drinking alcohol at increased or higher risk of harm is most common among adults aged 55 to 64. What percentage of UK men in this age group usually drink over 14 units in a week?

Select one:

- ☐ 4%
- ☐ 38%
- ☐ 15%
- ☐ 22%
- ☐ 30%

Your answer is incorrect.

Explanation:

In the UK, the proportion of adults usually drinking over 14 units of alcohol in a week varies across age groups. It is most common among men and women aged 55 to 64 (38% and 19%) respectively. Proportions drinking at these levels when declines among both sexes from the age of 65. Across all age groups, men are more likely to drink at increasing and higher risk levels than women.

Ref: <https://digital.nhs.uk/data-and-information/publications/statistical/statistics-on-alcohol/2020/part-4>

The correct answer is: 38%



Question 57

Not answered

Marked out of 1.00

What is the lifetime risk of suicide in dependent drinkers?

Select one:

- ☐ 1-2%
- ☐ 4%
- ☐ 10-15%
- ☐ 25%
- ☐ 40%

Your answer is incorrect.

Explanation:

Suicide in those with **alcohol dependence** is classically quoted as a **lifetime risk of 10-15%**. The lifetime risk of suicide in those with **alcohol problems** is estimated to be 4%. **Risk factors** include psychiatric comorbidity, social isolation, physical ill health, repeated failed attempts at abstinence.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 604.

The correct answer is: 10-15%

Question 58

Not answered

Marked out of 1.00

According to ICD -11 criteria, what is the duration of features required to make a diagnosis of alcohol dependence in a non-daily drinker?

Select one:

- ☐ 2 months
- ☐ 3 months
- ☐ 12 months
- ☐ 1 month
- ☐ 6 months

Your answer is incorrect.

Explanation:

In ICD-11, **features of alcohol dependence** are usually evident over a period of **at least 12 months**, but the diagnosis may be made if the alcohol use is **continuous (daily or almost daily)** for **at least 3 months**. ICD-11 alcohol dependence has the characteristic feature of a **strong internal drive to use alcohol** as manifested by:

1. **Impaired control** over alcohol use
2. **Increased priority** given to use over other activities
3. **Persistence of use despite harm or negative consequences**

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: 12 months

Question 59

Not answered

Marked out of 1.00

Cerebellar degeneration has been reported to occur in up to what percentage of alcoholics?

Select one:

- ☐ 50%
- ☐ 5%
- ☐ 33%
- ☐ 1%
- ☐ 75%

Your answer is incorrect.

Explanation:

Cerebellar degeneration is a pathologic term to describe slowly accumulating cerebellar changes caused by alcohol toxicity. Cerebellar degeneration occurs in alcoholics with and without micronutrient deficiencies and is considered the most common CNS complication, affecting an estimated 10% to 25% of alcoholics, with some studies citing a 33% incidence among alcoholics. Cerebellar ataxia is the clinical correlate to cerebellar degeneration. The characteristic presentation is a wide-based, ataxic gait and relative absence of appendicular ataxia or other cerebellar signs, such as nystagmus or dysarthria. Once cerebellar degeneration is established, aside from thiamine supplementation, treatment is largely supportive because the damage is irreversible.

Ref: Hammoud N, Jimenez-Shahed J. Chronic neurologic effects of alcohol. Clinics in Liver Disease. 2019 Feb 1;23(1):141-55.

The correct answer is: 33%

Question 60

Not answered

Marked out of 1.00

Mr Z suffers from alcohol dependence syndrome of moderate severity. He is being started on a chlordiazepoxide regime. The typical regime usually lasts for

Select one:

- ☐ 1-2 days
- ☐ 2-3 days
- ☐ 3-5 days
- ☐ 5-7 days
- ☐ 10-14 days

Your answer is incorrect.

Explanation:

In the 'reducing regime' management of alcohol withdrawal, benzodiazepines are prescribed in rapidly reducing doses in order to avoid development of secondary, iatrogenic dependence, while covering the period of maximum risk. Chlordiazepoxide may be preferred for outpatient use, as it has lower abuse potential. Diazepam is often preferred for inpatient use, as it is faster-acting and can be given parenterally. For moderate alcohol dependence, a typical regime might be 10-20mg chlordiazepoxide 4 times a day, reducing gradually over 5-7 days. Note that 5-7 days' treatment is adequate and longer treatment is rarely helpful or necessary.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 592-593.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 459.

The correct answer is: 5-7 days

Question **61**

Not answered

Marked out of 1.00

The starting dose of acamprosate in adults weighing 60 kg or more

Select one:

- ☐ 999 mg 3 times daily
- ☐ 333 mg 3 times daily
- ☐ 666 mg 3 times daily
- ☐ 300 mg 3 times daily
- ☐ 100 mg 3 times daily

Your answer is incorrect.

Explanation:

The dosing for acamprosate is 1998mg daily (666mg three times per day) for individuals over 60kg. For those under 60kg, the dose is 1332mg daily. Acamprosate should be initiated as soon as possible after abstinence has been achieved (the BAP consensus guidelines recommend that acamprosate should be started 'during detoxification' because of its potential neuroprotective effect). NICE recommends that acamprosate should be continued for up to 6 months with regular (monthly) supervision. The SPC (summary of product characteristics) recommends that it is given for 1 year.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 463.



Pharmacological treatments for relapse prevention of AUD

Naltrexone

- **Mu-opioid antagonist**
- 50mg daily
- **Cautions:** cirrhosis, patients on opioids
- **SE:** nausea, headaches, anxiety
- More effective in **reducing risk of lapse becoming a relapse**

Acamprosate

- Normalizes **NMDAr dysregulation**
- 666mg, 3 times/day if over 60kg (if <60kg dose is 1332mg daily)
- **Cautions:** cirrhosis, elderly, underweight
- **SE:** diarrhoea (usually settles within 7 days), pruritus, rash
- More effective to **maintain abstinence**



(c) Servier Medical Art

Disulfiram

- **Aldehyde dehydrogenase inhibitor**
- 200-250mg daily
- **Cautions:** suicidal patient, high cardiovascular risk
- **SE:** metallic taste, halitosis, interactions with alcohol, headache, peripheral neuropathy, liver damage
- More effective if **intake is witnessed**

- Naltrexone and acamprosate NNT vs. placebo 9-12
- Other options: Baclofen (GABA-B receptor agonist) and Nalmefene (opioid antagonist)
- All should be combined with psychosocial intervention
- No robust data for optimum length of pharmacotherapy exist

The correct answer is: 666 mg 3 times daily



Question **62**

Not answered

Marked out of 1.00

The onset of delirium tremens usually occurs how many hours after the last drink?

Select one:

- ☐ 24-48 hours
- ☐ 48-72 hours
- ☐ 72-96 hours
- ☐ 4-6 hours
- ☐ 12-24 hours

Your answer is incorrect.

Explanation:

The usual timing for onset of delirium tremens is 72-96 hours after the last drink. The classic triad of symptoms is clouding of consciousness/confusion, vivid hallucinations (particularly in visual and tactile modalities) and marked tremor. Other clinical features include autonomic hyperactivity (tachycardia, hypertension, sweating and fever), paranoid delusions, agitation and insomnia. Prodromal symptoms include night-time insomnia, restlessness, fear, and confusion.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 455.

The correct answer is: 72-96 hours

Question 63

Not answered

Marked out of 1.00

Which of the following is true about relapse prevention in alcohol dependence?

Select one:

- ☐ Acamprosate is not licensed for the elderly
- ☐ If a patient relapses on acamprosate it must be stopped at once
- ☐ Smoking is contraindicated when taking disulfiram
- ☐ Acamprosate and disulfiram must not be combined
- ☐ Naltrexone has no place in treating alcohol dependence

Your answer is incorrect.

Explanation:

Safety and efficacy of acamprosate have not been established in patients younger than 18 years or older than 65 years. Acamprosate should not be administered to children, adolescents, or the elderly.

Ref: <https://www.medicines.org.uk/emc/product/2729/smpc#gref>

The correct answer is: Acamprosate is not licensed for the elderly

Question **64**

Not answered

Marked out of 1.00

Which term refers to 'discrete episodes of anterograde amnesia that occur in association with alcohol intoxication'?

Select one:

- ☐ Alcoholic dementia
- ☐ Wernicke encephalopathy
- ☐ Korsakoff syndrome
- ☐ Alcoholic blackouts
- ☐ Pathological intoxication

Your answer is incorrect.

Explanation:

Alcohol induced amnesia (also called 'blackouts' or 'palimpsest') is not included in DSM-5/ICD-11. These are discrete episodes of anterograde amnesia that occur in association with alcohol intoxication. Patients will report a gap in their memory lasting for several hours, with global or partial amnesia for their actions during that time. The patient's behaviour is usually characteristic of their normal behaviour when intoxicated. There is a breakdown in transfer of information from short-term to long-term storage that occurs while immediate (very brief short-term) and remote (long-term; formed prior to intoxication) memory abilities remain intact. Alcohol facilitates faster forgetting. Rapid forgetting is a hallmark of hippocampal dysfunction.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 600.

Hermens DF, Lagopoulos J. Binge drinking and the young brain: A mini review of the neurobiological underpinnings of alcohol-induced blackout. Frontiers in psychology. 2018 Jan 19;9:12.

The correct answer is: Alcoholic blackouts

Question 65

Not answered

Marked out of 1.00

A 49-year-old man is heavily dependent on alcohol. Due to various physical problems he was strongly advised to abstain. Unfortunately since his last episode of intoxication he has developed recurrent voices that frighten him. In spite of 2 months of abstinence these hallucinations have continued. Which one of the following diagnoses best suits his presentation?

Select one:

- ☐ Delirium tremens
- ☐ Alcoholic hallucinosis
- ☐ Delusional disorder
- ☐ Depressive psychosis
- ☐ Schizophrenia

Your answer is incorrect.

Explanation:

Alcoholic hallucinosis is a substance-induced psychotic illness that is a rare complication of prolonged heavy alcohol use. In ICD-11, it is designated as 'alcohol-induced psychotic disorder with hallucinations'. It is characterised by experiencing hallucinations (usually auditory) while sober. It usually starts with elemental hallucinations (e.g. bangs or murmurings). With continued alcohol use they become formed voices, usually derogatory. In 95% of patients, symptoms rapidly resolve on ceasing alcohol consumption, but rapidly recur on restarting drinking; in 5%, symptoms persist with abstinence.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 602.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Alcoholic hallucinosis

Question **66**

Not answered

Marked out of 1.00

Which of the following medications is best avoided for outpatient assisted withdrawal from alcohol due to the risk of respiratory depression?

Select one:

- ☐ Carbamazepine
- ☐ Oxazepam
- ☐ Diazepam
- ☐ Chlormethiazole
- ☐ Lorazepam

Your answer is incorrect.

Explanation:

Chlormethiazole is not appropriate for use in outpatient assisted withdrawal because there is a high risk of misuse and overdose. It can have fatal consequences in overdose resulting from coma and respiratory depression, especially when taken with alcohol.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Chlormethiazole

Question **67**

Not answered

Marked out of 1.00

Choose the one feature that is not described under the Edwards and Gross criteria (1976) for alcohol dependence.

Select one:

- ☐ subjective awareness of compulsion
- ☐ repeated withdrawals
- ☐ narrowed repertoire
- ☐ reinstatement after abstinence
- ☐ decreased tolerance

Your answer is incorrect.

Explanation:

Original Edwards & Gross criteria for alcohol dependence (1976):

1. Narrowing of the drinking repertoire
2. Salience/primacy of alcohol-seeking behaviour
3. Increased tolerance
4. Repeated withdrawal symptoms
5. Relief or avoidance of withdrawal symptoms by further drinking
6. Subjective awareness of compulsion to drink
7. Reinstatement after abstinence

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 574.

Edwards G, Gross MM. Alcohol dependence: provisional description of a clinical syndrome. British medical journal. 1976 May 1;1(6017):1058.

The correct answer is: decreased tolerance

Question **68**

Not answered

Marked out of 1.00

Characteristic features of delirium tremens include all of the following except

Select one:

- ☐ Clouded consciousness
- ☐ Diarrhoea
- ☐ Hallucinations
- ☐ Fluctuant blood pressure and pulse
- ☐ Agitation

Your answer is incorrect.

Explanation:

Delirium tremens (DTs) is an acute confusional state secondary to alcohol withdrawal. It is often preceded by withdrawal symptoms and seizures may herald its onset. DTs occur in approximately 5% of episodes of alcohol withdrawal. Risk factors for DTs include a long history of alcohol dependence, past history of DTs, being older, being medically compromised and a poor nutritional state. DTs are characterised by a range of clinical symptoms including autonomic hyperactivity, marked agitation, clouding of consciousness, perceptual disturbances, and paranoid delusions. It has a mortality of 1-15%.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. pp. 590-591.

Sinclair JM, O'Neill A. Update on the management of alcohol use disorders. BJPsych Advances. 2020 Mar;26(2):82-91.

The correct answer is: Diarrhoea

Question 69

Not answered

Marked out of 1.00

Mr. Z has a history of seizures and is now admitted to a medical ward for alcohol withdrawal. Which of the following drugs should be prescribed in this situation?

Select one:

- ☐ Diazepam
- ☐ Oxazepam
- ☐ Clonazepam
- ☐ Phenytoin
- ☐ Carbamazepine

Your answer is incorrect.

Explanation:

A meta-analysis of trials assessing the efficacy of drugs preventing alcohol withdrawal seizures demonstrated that benzodiazepines, particularly long-acting preparations such as diazepam, significantly reduced seizures de novo. A long-acting benzodiazepine is recommended as prophylaxis in those with a previous history of seizures. Phenytoin does not prevent alcohol withdrawal-related seizures when used on its own or in combination with benzodiazepines.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 455.

The correct answer is: Diazepam

Question 70

Not answered

Marked out of 1.00

Acamprosate appears to act centrally on which of the neurotransmitter systems?

Select one:

- ☐ Serotonin
- ☐ Noradrenaline
- ☐ Dopamine
- ☐ Acetylcholine
- ☐ GABA

Your answer is incorrect.

Explanation:

Acamprosate is a derivative of the amino acid taurine and interacts with both the glutamate system to inhibit it and with the GABA system to enhance it, a bit like a form of 'artificial alcohol'. Thus, when alcohol is taken chronically and then withdrawn, the adaptive changes that it hypothetically causes in both the glutamate system and the GABA system create a state of glutamate overexcitement and even excitotoxicity as well as GABA deficiency. To the extent that acamprosate can substitute for alcohol in patients during withdrawal, the actions of acamprosate mitigate the glutamate hyperactivity and the GABA deficiency.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. p. 556.

The correct answer is: GABA

Question 71

Not answered

Marked out of 1.00

A 46-year-old gentleman has been started on disulfiram for the treatment of alcohol dependence. A common side effect reported by patients taking disulfiram is

Select one:

- ☐ Dystonia
- ☐ Urinary incontinence
- ☐ Halitosis
- ☐ Visual hallucinations
- ☐ Skin pigmentation

Your answer is incorrect.

Explanation:

Common side effects of disulfiram include halitosis and headache. There are rare reports of psychosis and hepatotoxicity. Disulfiram can act as a helpful adjunct to therapy and allow the patient's relatives/employers to regain confidence in their ability to remain abstinent. There is evidence for increased efficacy with supervised administration.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 598.

The correct answer is: Halitosis

Question **72**

Not answered

Marked out of 1.00

Which of the following medications used in the setting of delirium tremens may increase the risk of seizures?

Select one:

- ☐ Chlormethiazole
- ☐ Oxazepam
- ☐ Diazepam
- ☐ Haloperidol
- ☐ Carbamazepine

Your answer is incorrect.

Explanation:

In most cases of delirium tremens, antipsychotics should not be used, or used only after large doses of benzodiazepines have failed. NICE suggests that haloperidol can be used to manage behavioural disturbance in delirium tremens, but others urge caution in view of its cardiotoxicity and propensity to provoke seizures.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 472-473.

The correct answer is: Haloperidol

Question **73**

Not answered

Marked out of 1.00

Patients with Wernicke encephalopathy show gliosis and small haemorrhages especially in which of the following brain regions?

Select one:

- ☐ Prefrontal cortex
- ☐ Amygdala
- ☐ Golgi bodies
- ☐ Mamillary bodies
- ☐ Cerebellar peduncles

Your answer is incorrect.

Explanation:

The distribution of the lesions of Wernicke encephalopathy is characteristic and accounts for the symptoms that include disturbances of wakefulness, hypertonia, and ocular palsies. They are found in the periventricular areas, including the medial aspect of the thalamus, hypothalamus, and mamillary bodies; the periaqueductal region at the level of the third cranial nerve; the reticular formations of the midbrain, caudal portion of the corpora quadrigemina, and the floor of the fourth ventricle may also be involved. The mamillary bodies are the most frequently affected structures.

Ref: Gray F, Duyckaerts C, de Girolami U. Escourolle and Poirier's Manual of Basic Neuropathology, 6th Edition. 2019. p. 226.

The correct answer is: Mamillary bodies

Question 74

Not answered

Marked out of 1.00

The MATCH trial is a landmark study evaluating treatment options for alcohol dependence. Which of the following is an accurate finding from the MATCH trial?

Select one:

- ☐ Impulsivity is the strongest predictor of long term drinking outcome
- ☐ Binge drinkers do better with a CBT approach
- ☐ Matching patients to treatment options does not make a significant impact on outcome
- ☐ Younger drinkers do better with a motivational approach
- ☐ At least initially, drinkers with high psychiatric morbidity do better with CBT than 12 step programmes

Your answer is incorrect.

Explanation:

Matching is the concept of tailoring treatments to particular types of clients to increase effectiveness. There is little evidence that matching people with alcohol dependence to treatment approaches increases effectiveness. Project MATCH (Matching Alcoholism Treatments to Client Heterogeneity) by the NIAAA was the largest national multi-site RCT of alcoholism treatment matching. Clients were randomly allocated to cognitive behavioural coping-skills therapy, motivational enhancement therapy (MET), or 12-step facilitation (TSF). It revealed few matching effects. The UK Alcohol Treatment Trial (UKATT) later explored client-treatment matching in the treatment of alcohol use comparing MET with social behaviour and network therapy (SBNT). None of the five matching hypotheses were supported.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Matching patients to treatment options does not make a significant impact on outcome

Question 75

Not answered

Marked out of 1.00

The two interventions compared in the UK Alcohol Treatment Trial (UKATT) were

Select one:

- ☐ CBT vs. medication based management
- ☐ CBT vs. social behaviour and network therapy
- ☐ Motivational therapy vs. social behaviour and network therapy
- ☐ CBT vs. motivational therapy
- ☐ Motivational therapy vs. twelve step programme

Your answer is incorrect.

Explanation:

Matching is the concept of tailoring treatments to particular types of clients to increase effectiveness. There is little evidence that matching people with alcohol dependence to treatment approaches increases effectiveness. Project MATCH (Matching Alcoholism Treatments to Client Heterogeneity) by the NIAAA was the largest national multi-site RCT of alcoholism treatment matching. Clients were randomly allocated to cognitive behavioural coping-skills therapy, motivational enhancement therapy (MET), or 12-step facilitation (TSF). It revealed few matching effects. The UK Alcohol Treatment Trial (UKATT) later explored client-treatment matching in the treatment of alcohol use comparing MET with social behaviour and network therapy (SBNT). None of the five matching hypotheses were supported.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Motivational therapy vs. social behaviour and network therapy

Question **76**

Not answered

Marked out of 1.00

Which of the following statements concerning neuroimaging in Wernicke encephalopathy (WE) is NOT true?

Select one:

- ☐ MRI is better than CT for suspected WE
- ☐ MRI may show mamillary body lesions in WE
- ☐ MRI cannot predict progression to Korsakoff psychosis
- ☐ MRI is the most valuable method to confirm a diagnosis of WE
- ☐ MRI has high sensitivity but poor specificity in WE

Your answer is incorrect.

Explanation:

In Wernicke encephalopathy (WE), computed tomography (CT) can show areas of reduced attenuation density at the periaqueductal grey matter and the medial portion of the thalami but, in most cases, CT findings are negative in the acute phase of WE. MRI typically shows abnormal intensity alteration in bilaterally symmetrical lesions in the paraventricular regions of the thalamus, hypothalamus, mamillary bodies, periaqueductal region and floor of the fourth ventricle. MRI has a low sensitivity of only 53% but high specificity of 93% for the diagnosis of WE, thus MRI is currently considered the most valuable method to confirm the diagnosis.

Ref: Ota Y et al. Comprehensive review of Wernicke encephalopathy: pathophysiology, clinical symptoms and imaging findings. Japanese journal of radiology. 2020 Sep;38(9):809-20.

The correct answer is: MRI has high sensitivity but poor specificity in WE

Question **77**

Not answered

Marked out of 1.00

An opioid receptor antagonist that is effective in blocking the 'high' of alcohol is

Select one:

- ☐ Chlordiazepoxide
- ☐ Diazepam
- ☐ Acamprosate
- ☐ Naltrexone
- ☐ Disulfiram

Your answer is incorrect.

Explanation:

Naltrexone is a mu-opioid antagonist that hypothetically blocks the euphoria and 'high' of heavy drinking. This theory is supported by clinical trials that show that naltrexone reduces days of heavy drinking (defined as five or more drinks per day for a man and four or more for a woman) and also increases the chances of attaining complete abstinence from alcohol.

Ref: Stahl SM. Stahl's Essential Psychopharmacology: Neuroscientific Basis and Practical Applications, 5th Edition. 2021. p. 556.

The correct answer is: Naltrexone

Question 78

Not answered

Marked out of 1.00

The symptom/sign that responds earlier than others when thiamine is replaced in a patient with Wernicke encephalopathy is

Select one:

- ☐ Ophthalmoplegia
- ☐ Ataxia
- ☐ Neuropathy
- ☐ Apathy
- ☐ Amnesia

Your answer is incorrect.

Explanation:

With treatment of Wernicke encephalopathy, **ophthalmoplegia and confusion resolve within days**. Ataxia, neuropathy, and nystagmus may be prolonged.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 606.

The correct answer is: Ophthalmoplegia

Question 79

Not answered

Marked out of 1.00

Mr. Y was assessed by the drug and alcohol team, who recommended alcohol detoxification. He is a chronic smoker and has a history of a chronic respiratory disease. Which one of the following is the most suitable drug for detoxification?

Select one:

- ☐ Chlordiazepoxide
- ☐ Clonazepam
- ☐ Oxazepam
- ☐ Temazepam
- ☐ Diazepam

Your answer is incorrect.

Explanation:

In the setting of alcohol detoxification, **shorter-acting** benzodiazepines (e.g. oxazepam) may be preferred in those for whom **over-sedation must be avoided**, in people with **liver disease** who may not be able to metabolise long-acting agents efficiently, and in people with **COPD**. However, there is a greater risk of **breakthrough seizures** and **greater potential for misuse**.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on diagnosis, assessment and management of harmful drinking and alcohol dependence. Updated edition 2020.

The correct answer is: Oxazepam

Question 80

Not answered

Marked out of 1.00

Which of the following is important for the prophylaxis and treatment of Wernicke-Korsakoff syndrome?

Select one:

- ☐ Parenteral thyroid replacement
- ☐ Parenteral mineral replacement
- ☐ Parenteral albumin replacement
- ☐ Parenteral aldehyde replacement
- ☐ Parenteral vitamin replacement

Your answer is incorrect.

Explanation:

It is generally advised that patients undergoing inpatient alcohol detoxification should be given parenteral thiamine (vitamin B complex; in the UK – Pabrinex) as prophylaxis. The standard advice is one pair of Pabrinex IM high potency daily for 5 days, followed by oral thiamine and/or vitamin B compound for as long as needed (where diet is inadequate, or alcohol consumption is resumed). If Wernicke encephalopathy is suspected, the patient should be transferred to a medical unit where IV thiamine can be administered.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 461-462.

The correct answer is: Parenteral vitamin replacement



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